Find a problem people care enough to solve – HappyHealth – Home Medical Care, Karachi

Low urgency

Latent Pain

They notice it, but live with it.

HappyHealth example: Elderly parent in Gulshan needs BP check once a week. Family adjusts, calls neighbor. No search.
→ NOT our wedge. Latent pain, low willingness to pay.

Ongoing frustration

Passive Pain

It wastes time or creates hassle.

HappyHealth: DHA family spends 10 hrs/week coordinating unqualified maid, hospital queues for labs, medicine delivery fails.
Time cost Rs.4,500/day lost work + Rs.5k per hospital trip.
→ Ongoing frustration. Will pay if easy.

Actively looking

Active Pain

They are already searching for a fix.

HappyHealth – OUR WEDGE: Families in DHA/Clifton/PECHS WhatsApp groups: 'Need qualified nurse for father, diabetes 32.9% Karachi'. Overseas Pakistanis UAE/UK/US FB groups: 'Need trusted nurse for mother in Karachi'. Rozee, OLX searches, oladoc Rs.200-5k. They already pay Lajpal Rs.95k-120k/mo.
Proof: Human Healthcare 2022 started DHA/Clifton, 4.9/5 rating.
→ **ACTIVE PAIN – Our primary wedge**

Urgent crisis

Crisis Pain

Something important breaks; they need help now.

HappyHealth: Father discharged from AKU/Liaquat post-surgery, needs daily dressing + vitals + medicine at residence within 24h. Overseas son in Dubai must fly back or quit job. Equipment needed: bed, suction, ECG now.
Risk: Wrong medicine, bedsore, readmission cost Rs.150k.
→ **URGENT CRISIS – Second wedge – hospital discharge**

Problem Drilling (5-Why + JTBD Hybrid) – Applied to HappyHealth

Surface the Job	**When [situation], I want to [motivation] so I can [outcome].** **Job 1 – Overseas Son:** When my elderly father (70, diabetes 32.9% + HTN 30.4% Karachi) in DHA needs daily care and I am in Dubai UAE, I want to hire a PMDC-verified nurse who comes on time with vitals log so I can have peace of mind and avoid flying back / quitting job. **Job 2 – Local Daughter:** When my mother is discharged from AKU post-surgery and needs dressing + medicine at residence within 24h, I want to book a qualified nurse + equipment (bed, ECG) within 2 hours so I can avoid Rs.150k readmission and hospital queues. **Job 3 – Affluent Family:** When I already pay Rs.95k-120k/mo to unqualified maid (Lajpal benchmark) with no vitals log, I want to pay Rs.67,500 for 15 days to verified nurse with dashboard so I can pay less, nurse earn more, and see BP/sugar/medicine proof.
Find the Failure Point	**What is the hardest part of getting that job done today?** • **No verified pool:** Rozee, OLX, WhatsApp groups have nurses but no PMDC/PNC license check, no call to previous hospitals, no police verification, no CNIC – trust deficit. • **No on-time guarantee:** No zone hubs DHA/Clifton + Gulshan/PECHS, no GPS, no <2h SLA – Human Healthcare model missing. • **No documentation:** AKU CarePartner documents all services, but local maids give no vitals log, no medicine log, no family sign proof – family can't verify. • **No dashboard for overseas:** No Family Health Dashboard, no WhatsApp updates after visits, no 48h reminders, no yearly planner. • **Equipment loss risk:** Your handwritten risk – equipment damaged/lost, no deposit SOP. → **Wedge = Verified + On-time + Documented + Dashboard**

Drill with 5 Whys	**Keep asking why until you hit a root cause that is structural. That's your wedge.** **Why 1:** Why can't families in DHA/Clifton find qualified home care at residence? → Because existing providers (maids, Lajpal) don't verify PMDC license + previous hospital reference + police check. **Why 2:** Why don't they verify? → Because Rozee/OLX listing model has no verification SOP – anyone can claim RN. **Why 3:** Why no SOP? → Because SHCC Sec14(1) says all HCEs must be licensed, but home care enforcement is weak, no standard for home care documentation. **Why 4:** Why no documentation standard? → Because hospitals (AKU) document all services, but home care is informal – no EMR, no vitals log, no family sign proof required. **Why 5 (Root – Structural):** Why informal? → Because Karachi 17.15M, 96% mobile, 70% internet, but trust infrastructure for home care doesn't exist – no PMDC-verified badge, no senior doctor supervises first visit, no case navigator, no GPS + zone hub for <2h SLA. → **Structural wedge: Build trust infrastructure = PMDC check + call hospitals + police + senior supervises first visit (Human Healthcare) + zone hubs + dashboard + deposit SOP.**
Quantify the Cost	**How much time / money / risk does this create per week? Get a number (Rs.).** **Time Cost per family per week:** • 10 hrs/week coordinating unqualified maid, hospital trips, labs, medicine delivery × Rs.450/hr lost work (DHA affluent) = Rs.4,500/week • Hospital queue for vitals + labs: 1 trip × Rs.5,000 (Careem + token + time) = Rs.5,000/week **Money Cost per month:** • Unqualified maid Rs.95,000-120,000/mo (Lajpal RN benchmark) with no vitals log = Rs.105,000 avg • Hospital readmission risk due to missed medicine: Rs.150,000 one-time (AKU) • Equipment rental from market without deposit SOP: BP Rs.1,000/day × 30 = Rs.30,000 vs our Rs.500/day = Rs.15,000 loss **Risk Cost:** • Wrong medicine / bedsore / fall: Priceless + job loss risk for overseas son (Dubai ticket Rs.80,000 + 2 weeks unpaid leave Rs.100,000) • Equipment damage/loss: Your risk – 2% loss × Rs.80,000 inventory = Rs.1,600/month per family, without deposit Rs.7,500 × 10 = Rs.75,000 liability **Total Quantified Cost per family per week:** Rs.4,500 (time) + Rs.5,000 (hospital) + Rs.26,250 (Rs.105k/4) = **Rs.35,750/week** or **Rs.143,000/month** – willing to pay Rs.67,500 for 15 days (Rs.4,500/shift) to save this. → **Get a number: Rs.35,750/week – this is what they already lose.**
Test Priority	**If this were solved tomorrow, what would you do differently?** **Test for Active Pain:** • Would you pay Rs.67,500 advance for 15 days (Rs.4,500/shift) for verified nurse with vitals log + family sign proof + dashboard? • Would you cancel unqualified maid Rs.95k-120k/mo and switch? • Would overseas son in Dubai pay Rs.60k-100k/mo via bank transfer for dashboard + WhatsApp updates after visits + quarterly telecon? • Would hospital discharge manager at AKU refer patients if SHCC license + senior doctor supervises first visit? **Pass Mark (from Weekly Card):** 8 problem interviews, 80% say YES would pay @ Rs.4,500/shift, 4 nurses verified PMDC + reference + police, 1 hospital manager says would refer. **If YES to 80%:** KEEP number (problem interviews). If <50% YES: CHANGE segment (Gulshan vs DHA) or CHANGE bet (use Rozee.pk for nurses). **No behaviour change = no real pain:** They already pay Rs.95k-120k to unqualified, already search WhatsApp groups DHA/Clifton, already pay oladoc Rs.200-5k per visit – proves behaviour, proves pain. → **Priority Test: Ask 'If I give you verified nurse tomorrow with dashboard, will you pay Rs.67,500 advance today?'** Count only replies that meet pass mark (Three Rules).

No behaviour change = no real pain. HappyHealth: Families already pay Rs.95k-120k/mo to unqualified + search WhatsApp groups + pay oladoc Rs.200-5k + Chughtai HB Rs.550 free sampling. Overseas already pay Human Healthcare 4.9/5. Behaviour exists, so pain is real. Wedge = Verified + On-time (<2h) + Documented + Dashboard + Deposit SOP.